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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°157 /MVE-BDBV/10/18/2026

Report date: October 18, 2026 Publication date: October 19, 2026

HIGHLIGHTS (last 24 hours)

- Ninety-seven (97) new confirmed cases of Ebola Virus Disease (EVD), including 30 community deaths, have been reported. These cases originated in the provinces of North Kivu (48 cases), Ituri (42 cases), and Haut-Uélé (7 cases). In addition, 11 more deaths of confirmed cases were recorded in Ebola Treatment Centers (ETCs). No new health zones have been added to the Ebola Virus Diseas affected in the last 24 hours.
- A total of 500 new samples were analyzed; 97 of these samples tested positive for virusBundibugyo, representing a 19.4% positivity rate

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9552	4,602	48.2%	937	2,550	76.9%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé and South Ubangi

66 Health Zones affected out of 178 (37.1%)

Summary of key points from the last 24 hours

As of October 18, 2026, 97 new confirmed cases of Ebola virus disease (EVD) have been reported in three provinces: **North Kivu (48 cases), Ituri (42 cases)** and **Haut-Uélé (7 cases).** Among these cases, **30 community deaths** were reported. In addition, **11 confirmed deaths** were recorded in Ebola Treatment Centers (6 in Ituri, 4 in North Kivu and 1 in Haut-Uélé), bringing **the total number of confirmed deaths** reported during the period to 41.

In addition, 20 patients (12 in Ituri, 7 in North Kivu and 1 in Tshopo) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has accumulated **9,552 confirmed cases and 4,602 deaths, representing a case fatality rate of 48.2%.** Ituri remains the epicenter of the epidemic, with **72.9%** of cumulative confirmed cases and **43.3%** of new confirmed cases reported in the last 24 hours.

66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 18, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	42	6960	3,217	46.2%	28/36 (77.8%)
North Kivu	48	2084	1,176	56.4%	17/34 (50.0%)
Haut-Uélé	7	415	177	42.7%	7/13 (53.8%)
Tshopo	0	73	25	34.2%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	2	1	50.0%	1/11 (9.1%)
Total	**97**	**9,552**	**4,602**	**48.2%**	**66/178 (37.1%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 18, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6960**	**3217**	**46.2%**	**42**	**14**	**6**	**20**
Adja	11	1	9.1%				0
Aru	12	6	50.0%	1	0	0	0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	216	48	22.2%				0
Boga	2	2	100.0%				0
Bunia	1895	633	33.4%	14	4	1	5
Damascus	41	10	24.4%	1	1	0	1
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	38	12	31.6%				0
Komanda	188	113	60.1%				0
Lita	309	156	50.5%				0
Logo	14	9	64.3%				0
Lolwa	36	9	25.0%				0
Mahagi	4	1	25.0%				0
Mambasa	74	32	43.2%	6	3	1	4
Mandima	337	170	50.4%	7	3	2	5
Mangala	382	164	42.9%	3	2	0	2
Mongbwalu	715	343	48.0%	2	1	0	1

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Nia-Nia	331	191	57.7%	1	0	0	0
Nizi	827	298	36.0%				0
Nyankunde	134	41	30.6%	0	0	1	1
Rimba	19	5	26.3%				0
Rwampara	1167	403	34.5%	7	0	1	1
Tchomia	76	25	32.9%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2084**	**1176**	**56.4%**	**48**	**16**	**4**	**20**
Blessed	524	354	67.6%	18	13	0	13
Good	24	8	33.3%	1	1	0	1
Butembo	312	188	60.3%	1	0	1	1
Goma	1	0	0.0%				0
Kalunguta	79	30	38.0%	3	0	1	1
Katwa	644	393	61.0%	5	1	0	1
Kayna	3	2	66.7%				0
Kyondo	44	21	47.7%				0
Lubero	35	14	40.0%				0
Mabalako	75	30	40.0%	1	0	0	0
Alimbongo	5	3	60.0%				0
Manguredjipa	16	5	31.3%	1	0	0	0
Masereka	39	13	33.3%				0
Musienene	152	71	46.7%	1	0	0	0
Mutwanga	24	5	20.8%			0	0
Oicha	52	23	44.2%	2	0	1	1
Vuhovi	55	16	29.1%	15	1	1	2
Upper Uele	**415**	**177**	**42.7%**	**7**	**0**	**1**	**1**
Boma Mangbetu	45	19	42.2%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	108	46	42.6%	2	0	0	0
Pawa	104	56	53.8%	2	0	1	1
Rungu	5	3	60.0%				0
Wamba	149	51	34.2%	3	0	0	0
Tshopo	**73**	**25**	**34.2%**	0	0	0	0
Bafwasende	17	2	11.8%				0
Kabondo	9	1	11.1%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	33	14	42.4%				0
Mangobo	10	7	70.0%				0
Tshopo	2	0	0.0%				0
Wanie - Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0

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Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Wapinda	2	1	50.0%				
Total	**9552**	**4602**	**48.2%**	**97**	**30**	**11**	**41**

***These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.**

NA: Not Applicable.

Table 3. Status of alerts notified as of October 18, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	101	7	108	18	7	80	0	25	4
Bas Uélé	118	1	119	13	0	105	1	13	0
Ituri	1,054	66	1,120	115	66	939	0	175	109
North Kivu	1,703	77	1,780	168	75	1,535	2	243	114
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
North Ubangi	6	1	7	6	1	0	0	7	4
Tshopo	109	1	110	4	1	105	0	5	1
Total General	**3091**	**153 3 244**		**324**	**150**	**2,764**	**3**	**468**	**232**

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Figure 1. Evolution of cases by date of symptom onset over the last 21 days

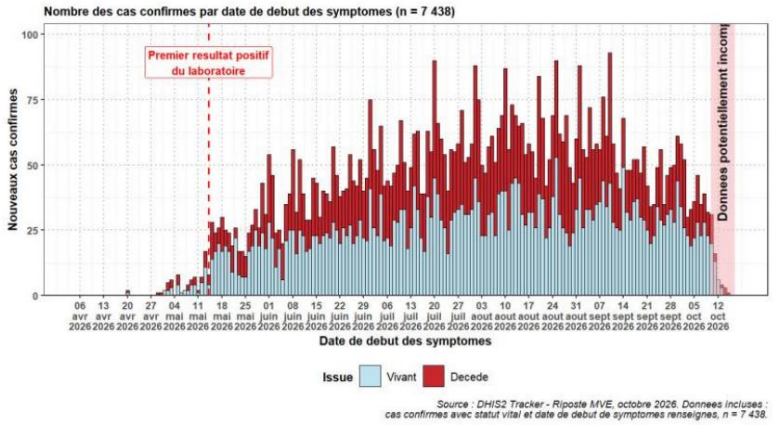
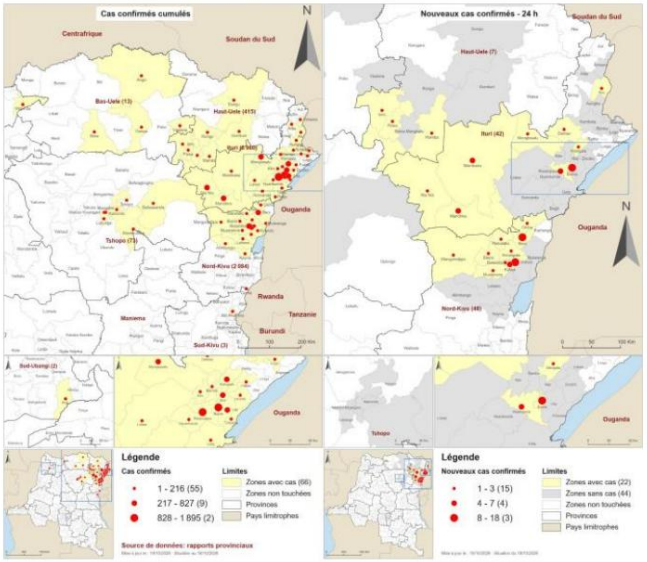


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

- During the day of October 18, 2026, 3,244 alerts were notified in the six provinces that submitted their data. Of these, 3,241 (99.9%) were verified and classified, with 474 (14.6%) validated as suspected cases and 2,767 (85.4%) invalidated. Three alerts remain to be clarified. Of the 474 validated suspected cases, 468 (98.7%) were investigated, with 232 (49.6%) transferred to COVID-19 treatment centers.
- Of the 32,744 contacts to be followed up in the seven provinces that submitted their data, 25,164 were seen in the last 24 hours, representing a follow-up rate of 76.9%, down from the previous day (80.0%) and below the required threshold of 95.0% (Table 4). Follow-up remains particularly low in Ituri (69.9%), notably in Bunia (26.0%; 936/3,599), and in North Kivu (78.6%).

2.1.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.1.3. Main actions

- On October 18, 2026, 6 alerts were notified to the PoE/PoC in the five monitored provinces: 5 in Ituri (1 at the Iga-Barrière PoC and 4 at the Biakato Mayi PoC) and 1 in North Kivu (PK5 PoC). These included 3 live alerts validated as suspected cases (2 in Ituri and 1 in North Kivu), of which 2 (66.7%) were transferred to a healthcare facility, the suspected case in Biakato Mayi having refused transfer; the 3 other alerts in Ituri were invalidated following secondary screening and no bodies were intercepted.

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Table 4. Status of key PoE/PoC indicators by province, as of October 18, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	98.4% (61/62)	90.5% (19/21)	55.6% (5/9)	100.0% (18/18)	80.0% (8/10)	92.5% (111/120)
% of functional strategic PoE/PoC	100%	100%	ND	100%	80%	ND
Number of people switched to PoE/PoC	141,229	89,733	11,157	19,136	17,106	278,361
% of travelers screened	97.8%	97.4%	98.9%	98.9%	98.8%	97.9%
% of travelers who washed their hands	97.4%	97.4%	98.9%	98.9%	98.8%	97.7%
% of travelers made aware	99.6%	97.4%	100.0%	100.0%	100.0%	99.0%
Number of alerts notified	5	1	0	0	0	6
Number of live alerts validated (suspected cases)	2	1	0	0	0	3
Number of bodies recovered today	0	0	0	0	0	0
% of suspected cases transferred to the CTE/CT	50.0%	100.0%	N / A	N / A	N / A	66.7%
% of lifeless bodies swabed/secured	N / A	N / A	N / A	N / A	N / A	N / A
% of suspected cases investigated within 4 hours	100.0%	100.0%	N / A	N / A	N / A	100.0%

2.1.4. Challenges

- **In Ituri,** insufficient mobility for supervision in the health zones of Rwampara, Nizi, and Bunia, delayed payments to service providers, and the need for an ambulance to serve the points of control (PoCs) in the Mandima health zone persist; traveler mobility at the Mudzi Bala PoC was disrupted by a drivers' protest, and the Avakubi PoC remains non-functional due to insecurity. In North Kivu, the completeness rate has fallen to 90.5% (19/21); the Butuhe PoC remains closed since the security incident of September 26, 2026, and the Kyavinyonge PoE has not reported; the security threat on the Butembo axis, shortages of supplies, and the lack of an ambulance dedicated to the PoCs persist; Kalundu Port SNCC and Baraka lack isolation facilities. In Haut-Uélé, the Doko PoE (no scheduled flights) and the Gossamu PoC (poor connection) did not report any revenue. In Bas-Uélé, the service providers for the PK12 Isiro PoC are on strike due to non-payment, and PNHF services, including training and motivation of service providers, water supply, mobility, connectivity, and the digitalization of PoE/PoC, are lacking.

2.2. Laboratory

2.2.1. Main actions

- **Ituri: 42 new positive results** (28 living and 14 dead) out of 202 samples received and analyzed (positivity: 20.8%).
- **North Kivu: 48 new positive results** (32 living and 16 dead) out of 247 samples received and analyzed (positivity: 19.4%).
- **Haut-Uélé: 7 new positive results** (7 living and 0 deaths) out of 28 samples received and analyzed (positivity: 25%).
- **Tshopo: No new positive results** on 11 new samples received and analyzed; all were revealed negatives
- **Bas Uélé: 12 new samples received at the Buta laboratory ,** including 4 new samples that were All those sent to the Isiro laboratory came back negative.
-

2.3. Infection Prevention and Control (IPC/EDS)

2.3.1. Main actions

- **Ituri :** 20 out of 26 rings were opened (76.9%), 17 out of 29 households were decontaminated (58.6%), and 6 out of 6 health facilities were decontaminated (100.0%). 44 blood tests were carried out out of 76 blood test alerts (57.9%), and 51 out of 67 bodies were swabed (76.1%).
- **Tshopo :** 7 ESS were monitored in 4 health zones and 72 providers were briefed on standard precautions, triage and screening; the Neema health center was decontaminated around the 72nd confirmed case and 10 PPLs were assessed there (5 to

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high risk, 3 at intermediate risk and 2 at low risk); 1 ring was opened in Makiso-Kisangani. Three EDS alerts received.

- **North Kivu :** 6 out of 16 testing centers (37.5%) were opened, 6 out of 8 health centers (75.0%) were decontaminated, and 5 out of 27 households (18.5%) were treated. None of the 5 decontaminated households received kits; no people with HIV were infected among the 14 confirmed reference cases. Of the 55 COVID-19 safety alerts received and 47 reports from the previous day, 62 COVID-19 tests were carried out and 55 bodies were swabbed, with no failed sampling attempts.
- **Bas-Uélé :** 9 ESS were monitored in 4 ZS (Buta 4/4, Ganga 1/1, Poko 1/1 and Viadana 3/3) and, in Ganga, 37 out of 39 providers (94.8%), 14 hygienists and 5 IPC supervisors were briefed; the DLM of the HGR of Buta (13/13), of the CS Makala (3/3), of the CM Mangindanginda (1/1) and of the CS of Congo (1/1) are functional.

2.3.2. Challenges

- **In Ituri,** 6 out of 26 rings remain to be opened, 12 out of 29 households have not been decontaminated, 21 EDS have failed and 16 out of 67 bodies (23.9%) could not be swabed.
- **In North Kivu,** decontamination teams were attacked in the health districts of Butsili and Butanuka (Beni health zone); 10 out of 16 decontamination rings remain to be opened, 22 out of 27 households have not been decontaminated, no households have received kits (0/5), and 40 decontamination efforts have been postponed. In Tshopo, the shortage of IPC supplies and equipment, as well as the lack of trained IPC personnel, persists, and the absence of families is preventing the completion of swabs.
  In Bas-Uélé, 16 out of 20 priority ESSs do not have functional triage, none have mattresses with fillings, 17 out of 20 do not have water storage covering 72 hours, waste management areas and hygienic latrines are lacking and the reporting of community death alerts remains low.

2.4. Holistic care

2.4.1. Main actions

- **In Ituri,** 104 new admissions were recorded against 68 discharges: 12 recoveries thus: 429 patients are hospitalized representing an occupancy rate of 41%, including 59 critical patients; 31 patients were evacuated by ambulance, 410 out of 418 patients (98%) received hot meals and 100 bags of blood were received from the CNTS in Kinshasa (103 in stock).

- **In North Kivu,** 74 new admissions were recorded, compared to 95 discharges: 7 recoveries (2 in Katwa and 5 in Butembo). Thus, 450 patients are hospitalized, including 62 children under 5 years old; among them, 2 patients are in critical condition. The overall occupancy rate of Ebola Treatment Centers (ETCs) is 87.6% (359/410), 93.8% for suspected cases (180/192), and 82.1% for confirmed cases (179/218). 91 patients (20.2%) are being treated in community health centers (ESS) that are not ETCs. 9 ambulance runs were successfully completed (7 out of 14 ambulances used), and 181 confirmed cases, 216 suspected cases, 427 people with HIV/AIDS (PPIs), and 197 caregivers received hot meals.

- **In Tshopo,** 2 new admissions of suspected cases were recorded at the Météo treatment center, compared to 3 discharges and 1 patient declared recovered. Thus, 16 patients are hospitalized out of 36 beds, representing an occupancy rate of 44.4%; 7 patients have been lost to follow-up. Since the beginning of the outbreak, 45 confirmed cases have received the experimental treatment (3 MBP134 and 42 Remdesivir) and 481 people have received post-exposure prophylaxis.

2.4.2. Challenges

- **In Ituri,** the low capacity of some CTE/CT (Kilo, Lolwa and Biakato), the care of 59 critical patients, the insufficient quantity of blood in relation to the needs of the CTE and hospitals, the lack of food for 8 patients in the Mandima health zone (CT Mandima and CT Makeke) and the low reporting of ambulance movements (18/33) remain worrying.

- **In North Kivu,** the beds for suspected cases are 93.8% full (180/192), 20.2% of hospitalized patients (91/450) are being treated outside of CTE/CT and the completeness of the data is only 71% (Beni, Musienene and Kalunguta missing).

- **In Tshopo,** the lack of food and nutritional support at the Bafwasende CT and the high number of people lost to sight (7, including 6 confirmed from Bafwasende and one confirmed motorcyclist who is wandering in the community) remain a concern.

- **In Bas-Uélé,** the delay in setting up the standard CTE, still under construction at the HGR of Buta, the lack of an ambulance, the failure to take into account nutritional support for isolated suspects in ESS and the delay in psychosocial support for the 3 confirmed cases remain major challenges.

2.5. Vaccination

2.5.1. Main actions

- **In Ituri,** vaccination of people living with HIV (PLHIV) using the Ervebo vaccine and research activities are continuing (Partners, EBO-PEP, and Oracle trials, preparations for the TUMAINI project); 57 members of the core team, registered nurses, and representatives of the PRECODESA (Regional Health Centers) in the Rwampara Health Zone were briefed on the clinical trial of the Bundibugyo virus vaccine, reserved for new contacts of confirmed cases. The cumulative number of vaccinated PLHIV remains at 1,260, as today's data has not yet been submitted.
- **In North Kivu,** the cumulative number of vaccinated PPLs in the Beni health zone reached 322 out of a target of 7,105 (4.5%); vaccination of PPLs in the Mabakanga health zone continued under supervision, and the availability of vaccine transport devices (ARKTEK) is being monitored with the WHO. In Bas-Uélé, 500 doses of Ervebo vaccine were

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Received, the deployment of a compassionate vaccination support team in the Ango health zone is in preparation and coverage reaches 11.4% of the target (1,500/13,202), including 21.6% in Buta and 20.5% in Ganga.

2.5.2. Challenges

- **In Ituri,** the target for PPL and TPL remains to be redefined for the calculation of vaccination coverage (1,120 vaccinated) for a provisional target of 1,000).
- **In Bas-Uélé,** the start-up of the UCCs is not effective, vaccines are not available in sufficient quantity and vaccination funds are still pending; in Tshopo, a discrepancy of 551 vaccinated persists between the Excel and DHIS2 databases and the Bafwasende health zone has not reported for four days; in North Kivu, the vaccination team lacks means of mobility (2 vehicles) and 3 small tents, and the vaccine transport devices (ARKTEK) remain to be confirmed with the WHO.

2.6. Continuity of Care

2.6.1. Main actions • In North Kivu, 5

out of 5 separated children were taken into care at the nursery, 16 out of 16 separated or orphaned infants are on breast milk substitute, 1 out of 1 malnourished child was taken into care, 8 confirmed unstable children benefited from specific diets and 36 patient households received food.

- **In Tshopo,** a 22nd frontline provider was confirmed at the Neema Health Center (Makiso-Kisangani), where the assessment noted the absence of a triage area, PPE and a functional incinerator, 2 out of 16 agents vaccinated (12.5%) and only one nurse trained on Ebola; the Central Prison Headquarters Medical Center was assessed and its recovery plan developed, and the monitoring of PPLs at the CTM and CTE as well as of recovered patients with complications at the Makiso-Kisangani General Referral Hospital continues.

- **In Bas-Uélé,** the evaluation of the Ganga General Referral Hospital shows a decrease in the service utilization rate, from 29.5% in June to 20.3% in September 2026, compared to a standard of 60%, reflecting low attendance or late recourse to care.

- **In Ituri,** 75 children received nutritional support from LNPE/SLM: 11 out of 11 at the CTE, 4 out of 4 at the crèche and 60 out of 60 in the community.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **In Ituri,** 20,347 home visits reached 72,604 people; 365 educational talks brought together 10,167 people; 82 mass awareness campaigns reached 3,019 people; 9 community debates brought together 345 people; and 8 advocacy efforts were organized on behalf of 30 religious and community leaders (Rwampara and Komanda). In Mandima, 276 students and teachers at the Biakato Institute were sensitized on Ebola virus disease prevention and reporting alerts in schools. A total of 304 community alerts, including 92 deaths, were reported; 49 out of 65 reports (75.4%) and 5 out of 7 incidents (71.4%) were resolved within 48 hours. The CREC facilitated the pre-listing of 216 contacts, the decontamination of 7 out of 7 households, the completion of 11 out of 11 EDS, the announcement of 17 out of 18 positive results and the transfer of 33 out of 33 suspects to the CTE; the completeness of CREC reports remains low (10/28 ZS, 36%).

- **In North Kivu,** 18,328 home visits reached 59,822 people, 179 educational talks were organized for 6,119 people, 120 mass communications reached 7,889 people, and 4 briefings reached 180 people; 562 community alerts were reported out of 650 expected (86.5%), 9 out of 13 feedback requests were received (69.2%), and 3 out of 8 incidents (37.5%) were resolved within 48 hours. The 5 refusals to transfer to the Butembo Treatment Center remain unresolved, and 3,467 out of 4,750 people (73.0%) were engaged in community actions. A dialogue on resistance was held with 30 local leaders from the Kasanga Community Action Group (Beni), and 44 members of the Beni Rapid Response Team received training.

- **In Tshopo,** the CREC supported the discharge of the newly recovered patient at the Météo CTE and activities surrounding case #72 in the Neema health district (contact tracing, community adherence to prophylaxis, and decontamination); awareness campaigns in religious communities in Kisangani, Bafwasende, and Wanie-Rukula reached approximately 44,153 people, with 2 out of 2 refusals overcome and 2 feedback issues resolved. In Bas-Uélé, 539 home visits reached 1,449 people, 2 educational talks reached 38 people, and 6 mass awareness campaigns reached 1,714 people in Ganga, Viadana, and Likati, with 5 out of 5 feedback issues resolved in Ganga.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- **In Ituri,** 13 confirmed cases out of 27 (48.1%), 93 confirmed cases under follow-up out of 156 (59.6%), 38 new suspects out of 104 (36.5%) and 108 suspects under follow-up out of 146 (74.0%) benefited from SMSPS interventions.
  The teams supported 213 caregivers and 63 PPLs, followed up on 14 out of 19 (73.7%) healers in the community and 13 out of 13 children in daycare, supported 8 out of 13 separated and orphaned children's households, reintegrated 5 out of 12 healers and announced 37 results, including 13 positive ones; 44 children are being followed and the training of 12 psychologists and APS/TPS (8 men and 4 women) has started in Mandima.
- **In Tshopo,** 3 out of 3 confirmed cases under follow-up, 2 out of 2 new suspected cases, and 5 out of 5 suspected cases under follow-up (100%) benefited from SMSPS interventions; 2 households received support for the actions

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public health, 6 PPLs supported, 1 resistance out of 1 broken and 1 negative result announced, and the APS of Bafwasende were coached.

- **In Bas-Uélé,** 73 people (67 men and 6 women) participated in psychoeducation sessions on EVD, GBV and child protection in the community and at checkpoints; no other SMSPS indicators were reported.

2.8.2. Challenges

- **In Ituri,** only 14 of the 28 affected health zones are covered (50.0%) and only 13 of the last 27 confirmed cases (48.1%) have been treated; the deployment of trained psychologists and APS/TPS is expected after the training.

- **In Bas-Uélé,** the psychosocial support for the 3 confirmed cases is lagging behind; in Tshopo, the APS of Wanie-Rukula still need to be trained.

2.9. Logistics
2.9.1. Main actions.

- Ensure the mobility of all field teams in the affected provinces; in North Kivu, the construction of the cabins at PoC PK5 and the passenger footbridge at Beni airport continues with the support of IOM; in Bas-Uélé, the construction of the CTE at the HGR in Buta and the rehabilitation of the DPS offices continue and the inputs for the ZS in preparation are being packaged.

2.10. Security
2.11. Main actions.

- In Tshopo, public order was restored at the La Grâce Medical Center (Beni) following resistance from a suspected case, and decontamination teams were attacked in the health districts of Butsili and Butanuka. Also in Tshopo, threats against teams are hindering investigations into cases 65 to 70 in Bafwasende; in Ituri, the Avakubi Point of Contact remains non-functional due to insecurity.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)
2.12.1. Main actions

- In North Kivu, 40 community leaders and radio agents (31 men and 9 women) were trained on PSEA/HS in Butembo and Katwa, and 406 people were sensitized on reporting via 495555/151 in Beni, Katwa, Nyiragongo, Kalunguta and Mabalako; in Tshopo, the bi-monthly work plan of the PEAHS pillar was developed.

2.12.2. Challenges

- At the national level, coverage of affected areas remains low (56 health zones out of 151 with PSEA activities, i.e. 37.1%), visibility support is insufficient and the effectiveness of CBCM is low; in Tshopo, collection tools and the mobility of focal points are lacking.

KEY MESSAGES

ÿ In the past 24 hours, 468 out of 474 suspected cases (98.7%) have been investigated in the provinces that submitted their data.
However, six suspected cases remain uninvestigated in Ituri (five in Nyankunde and one in Mongbwalu). Furthermore, the contact tracing rate has declined from 80.0% the previous day to 76.9%, remaining below the required threshold of 95.0%. This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risk of community transmission.

Maintain a level of vigilance regarding epidemiological surveillance, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care facilities with adequate capacity.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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